Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Joints and muscles

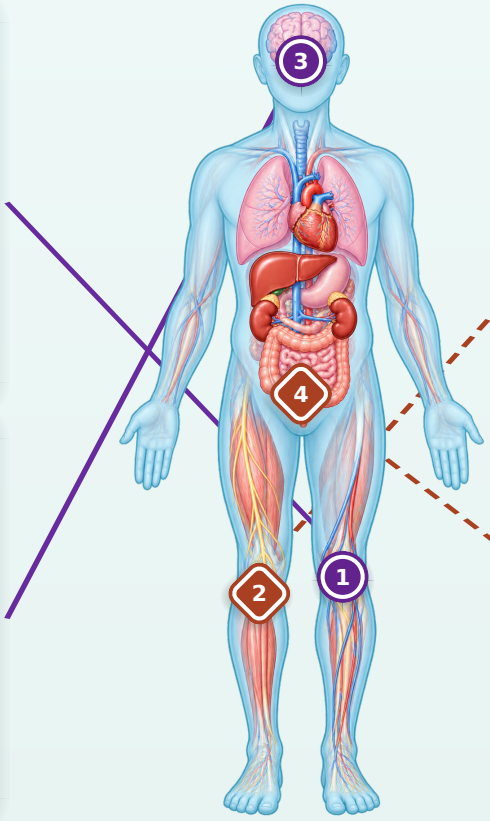
Letrozole

Pain or stiffness can affect activity; report persistent or limiting symptoms.

2 Bone strength

Letrozole

Bone loss may be silent. Your risk assessment and bone-density plan matter.



3 Hot flushes / sleep

Letrozole

Warm flushes and disrupted sleep can be treated; tell the team.

4 Vaginal / sexual symptoms

Letrozole

Dryness or discomfort deserves support. Ask before using hormone-containing treatments.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report persistent limiting joint pain, new severe bone pain or a possible fracture, jaundice, or new vaginal bleeding. New breast changes during preoperative treatment need review rather than waiting for the planned end date. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

Take with or without food. Take a forgotten dose when remembered unless the next dose is within about two to three hours; then skip it. Do not take two doses together.

Checks and follow-up

FBC, renal and liver profile; confirm menopause with hormone tests when uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. FBC, renal / liver profile and lipids when clinically indicated. Review bone density during treatment and after treatment according to bone risk. Neoadjuvant response and surgical planning need explicit review milestones.

The whole-treatment map distinguishes early-breast follow-up from advanced-disease response assessment.

Support between reviews

Symptoms and daily life

Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.

Support medicines

No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.

Tests and appointments

Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report persistent limiting joint pain, new severe bone pain or a possible fracture, jaundice, or new vaginal bleeding. New breast changes during preoperative treatment need review rather than waiting for the planned end date.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Your complete treatment reference

https://sactcheck.com/patient/00371/

Public information only. No personal information is encoded.